AI analysis · Based on Claude self-analysis and deep research

The feedback group: does Aurelia actually help people get better?

Thirty people, each with one issue Aurelia is meant to help with, will use it for eight weeks. This document explains why we run the study, how it is designed, what data we must collect, what counts as success, and the new Experimentation section in admin where the team tracks it.

Purpose: MVP validation **Study id:** fg-2026-01 **Admin:** Experimentation → Feedback group study (staging)

Prepared by: Claude (Anthropic's AI), for the Aurelia team

People in the feedback group

30

DESIGN six cohorts of 4–6

Length of the study

8 weeks

DESIGN baseline, then weeks 1–8

Main question asked every two weeks

WHO-5

DESIGN well-being, 0–100

Pass line for well-being

≥ 50%

DESIGN improve by 10+ points

In one paragraph. So far we have tested whether people *understand and come back to* Aurelia: a simulation with six AI personas, then three real interviews. We have not tested whether it *helps*. This study does that in the simplest honest way. Thirty people with one target issue each (bad sleep, anxiety, mental fog, low energy, poor focus, a stressed body) use Aurelia for eight weeks. Every two weeks they answer a short, widely used well-being questionnaire (WHO-5), and at the start, middle and end a validated questionnaire for their own issue. The app records how often they actually use it. Before we start we fix the pass lines, so the result cannot be read to fit. The study can show whether people improve while using Aurelia and whether they keep using it, which is enough to decide whether the MVP deserves a proper controlled trial. It cannot prove that Aurelia caused the change, and we will not claim that it did.

1. Why this study, and why now

The brief: a group of 30 people with different issues tries Aurelia for a period of time, to see whether they can "solve and win them": bad sleep to good sleep, mental fog to mental clarity. The admin needs a section to track and measure the group, as validation of therapeutic outcomes for the MVP.

WHAT WE HAVE	WHAT IT TOLD US	WHAT IT CANNOT TELL US
Retention simulation (25 Oct), six AI personas	Where first-time users get lost, and which fixes should bring them back	Anything about real people's outcomes; its numbers are predictions
Three real user interviews	What feels untrustworthy, what they want (words, not scores; the why; their wearable)	Whether using Aurelia for weeks changes anything for them
The app (production v1 / v2)	A one-tap check-in after each session, a five-chapter journey, a journal	Validated outcome measures; the check-in is a mood tap, not a clinical measure

The study fills the gap between "people like it" and "it helps". It is also the first time real use is measured over weeks. That is why the work to keep people signed in (decision D2) becomes a must-have before week 0.

2. The design at a glance

ELEMENT	CHOICE	WHY
Type	Single-arm, pre-registered pilot (everyone uses Aurelia; no control group)	The right size for an MVP. It tells us whether a controlled trial is worth running. A control group of 15 would be too small to show anything.
People	30 adults, recruited for one target issue each, never having used Aurelia	Matches the six goals onboarding already offers, so each person uses the product as it is
Length	8 weeks: baseline (week 0), then weeks 1–8	Long enough for sleep and stress to change; short enough to keep people. The brief asked for two months.
What they do	Use Aurelia as they like, with a suggested rhythm of 3+ sessions a week and the five-chapter journey	We test the real product, not a lab version. Adherence is measured, not forced.
Main measure	WHO-5 Well-Being Index at weeks 0, 2, 4, 6, 8	Five questions, free to use, validated in many languages, with a known meaningful change (10 points)
Issue measure	One validated questionnaire per cohort at weeks 0, 4, 8	"Bad sleep to good sleep" needs a sleep measure, not only general well-being
Safety	PHQ-9 at screening, week 4 and week 8; same-day human follow-up on any risk answer	People with real difficulties will join; we must notice when someone is getting worse
Identity	Pseudonymous ids (P01–P30) in every tool; names only with the research team	Health data. The admin never needs to know who someone is to measure the study.

3. Six cohorts, one measure each

Each cohort is one of the goals people already choose in onboarding. Each has a validated questionnaire for its issue, and the change that counts as meaningful for one person.

COHORT	APP GOAL	MEASURE	RANGE	MEANINGFUL CHANGE	PLACES
Bad sleep → good sleep	Sleep better	Insomnia Severity Index (ISI)	0–28	6 points lower	6
Anxious → calm	Feel calmer	GAD-7	0–21	4 points lower	6
Mental fog → clarity	Clear my mind	PROMIS Cognitive Function 4a (T-score)	20–80	5 points higher	5
Low energy → energy	More energy	PROMIS Fatigue 4a (T-score)	30–80	5 points lower	5
Distracted → focused	Focus	PROMIS Cognitive Function 4a (T-score)	20–80	5 points higher	4
Stressed body → regulated	Regulate my body	Perceived Stress Scale (PSS-10)	0–40	4 points lower	4

The meaningful-change figures are the thresholds commonly used in the research literature for these instruments. A clinical advisor should confirm them before week 0, together with licensing: WHO-5, PHQ-9, GAD-7 and PROMIS are free to use; ISI and PSS-10 may need permission for commercial use. Sleep and calm are the largest cohorts because they had the largest shares in the simulated panel. That is an assumption about early users, not a measurement.

4. The eight weeks

Week 0 Consent, screening (PHQ-9), WHO-5, cohort measure, interview 1, onboarding

Week 1 Use Aurelia; check-ins; sleep diary for the sleep cohort

Week 2 WHO-5

Week 3 Use; weekly one-liner

Week 4 WHO-5, cohort measure, PHQ-9, interview 2 · interim read

Week 5 Use

Week 6 WHO-5

Week 7 Use

Week 8 WHO-5, cohort measure, PHQ-9, value question, interview 3 · final read

Every session: a before-play tap and the existing "How do you feel now?" check-in. Daily, if the person agrees: wearable sleep, resting heart rate and HRV. Any time: "Something felt wrong", reviewed by a person.

5. The data we need to collect

This is the data the brief asked for. The last column says whether production can collect it today. Most of it has to be built before week 0 (section 9).

MEASURE	WHY WE NEED IT	WHEN	IN THE APP TODAY
WHO-5 well-being, 0–100	The one comparable outcome across all six issues; the primary result	Wk 0, 2, 4, 6, 8	NO
Cohort measure (table above)	Whether their own issue improved: "bad sleep to good sleep"	Wk 0, 4, 8	NO
PHQ-9 safety screen	To notice anyone getting worse; item 9 triggers a same-day call	Screening, wk 4, 8	NO
After-session check-in 1–5	Short-term effect of each session	Every session	YES needs saving to the account
Before-session state 1–5	Turns the check-in into a before/after for each session	Every session	NO
Sleep diary (sleep cohort)	Time to fall asleep, night wakings: the details ISI summarises	Every morning	NO
Wearable sleep, resting heart rate, HRV	An objective signal next to what people report; all three interviewees wear one	Daily, if consented	PARTLY v2 asks; connection needs backend
Engagement: sessions played, minutes, chapters, journal	Without use there is no effect to measure; it also separates "did not help" from "did not use"	Continuous	PARTLY views and taps only
Weekly one-liner and **interviews**	The why behind the numbers	Weekly; wk 0, 4, 8	NO research team
Value question (Sean Ellis) and would-recommend	Would they miss it: the product-market-fit signal	Wk 8	NO
Adverse events	Anything that felt worse, at any time	Any time	NO

6. What counts as success

Set before week 0 and not changed afterwards. Outcome lines are judged only at week 8; the admin shows interim figures as "not yet".

QUESTION	PASS LINE	WHY THIS LINE
Does well-being improve meaningfully?	≥ 50% of people gain 10+ WHO-5 points by week 8	10 points is the change the WHO-5's authors call meaningful; half the group is a strong signal for a pilot
Does their own issue improve?	≥ 40% reach the meaningful change on their cohort measure	Issue measures move more slowly than general well-being
Do people stay?	≥ 70% still enrolled at week 8	Below that, the people who remain are not representative
Do people actually use it?	≥ 60% play 3+ sessions in at least 60% of weeks	An effect without use would be noise; use without effect is its own finding
Do we have the data to judge?	≥ 85% of due questionnaires answered	Missing data weakens every other line
Is anyone harmed?	No serious adverse event; every alert followed up the same day	Non-negotiable in a mental-health-adjacent product
Would they miss it?	≥ 40% "very disappointed" (Sean Ellis)	The usual product-market-fit bar, also used in the retention test plan

7. How we will read the results

- **Responder rates with their uncertainty.** With 30 people, a 50% responder rate has a 95% range of roughly 32–68%, so we report the range, not just the number.
- **Change from baseline** on WHO-5 and each cohort measure: the average change, a paired test (Wilcoxon signed-rank), and an effect size.
- **Cohorts are described, not tested.** With 4–6 people each, a cohort result is a direction, not a finding.
- **Everyone counts:** people who withdraw stay in the denominator ("intention to treat"). We also report the result for people who used it as suggested.
- **Use is related to change:** do people who played more improve more? This is the closest a single-arm study gets to a dose signal.
- **What we will not claim:** that Aurelia caused the change, that it treats any condition, or any number in marketing. People who join a study to feel better often do; regression to the mean and expectation both push scores up. A controlled trial is the next step if the pass lines are met.

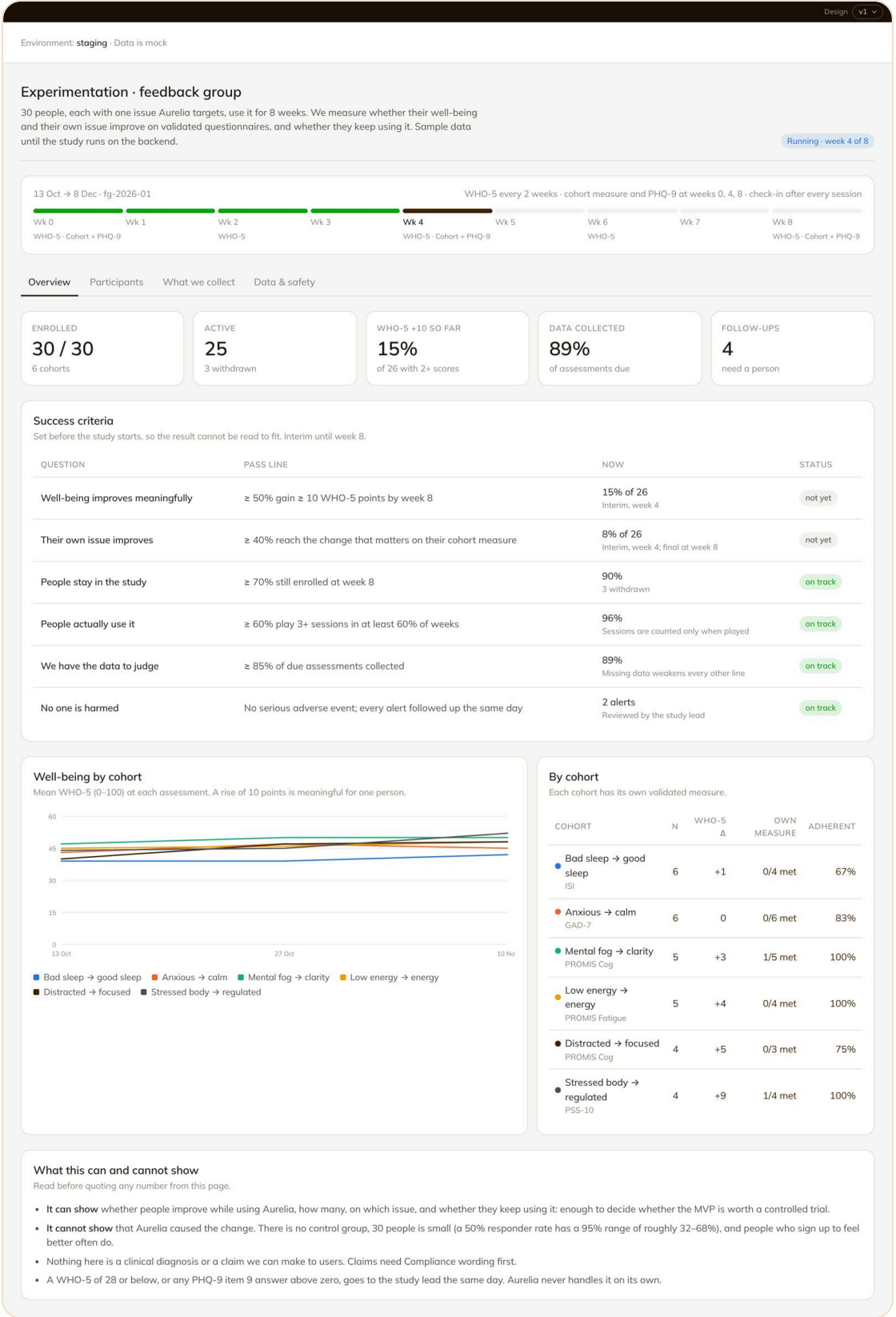
8. Safety, consent and privacy

- **Informed consent** before any data is stored: what is collected, who sees it, the right to withdraw at any time, and that Aurelia is not a treatment. Anyone in treatment keeps it; Aurelia is never a replacement.
- **Who should not join:** people in active crisis, with recent psychiatric hospitalisation, or scoring 20+ on PHQ-9 at screening. They are offered support resources instead.
- **Risk answers go to a person:** a PHQ-9 item 9 above zero, a WHO-5 of 28 or below, or a reported adverse event goes to the study lead, who calls the same day. Aurelia never handles it on its own, and the participant never sees a PHQ-9 score.
- **Privacy:** pseudonymous ids in every tool, journal text never leaves the participant's account without consent, and wearable data only with a separate consent.
- **Review:** an independent ethics review and a clinical advisor before week 0 are strongly recommended. They are needed if the results are ever to be published or used with partners.

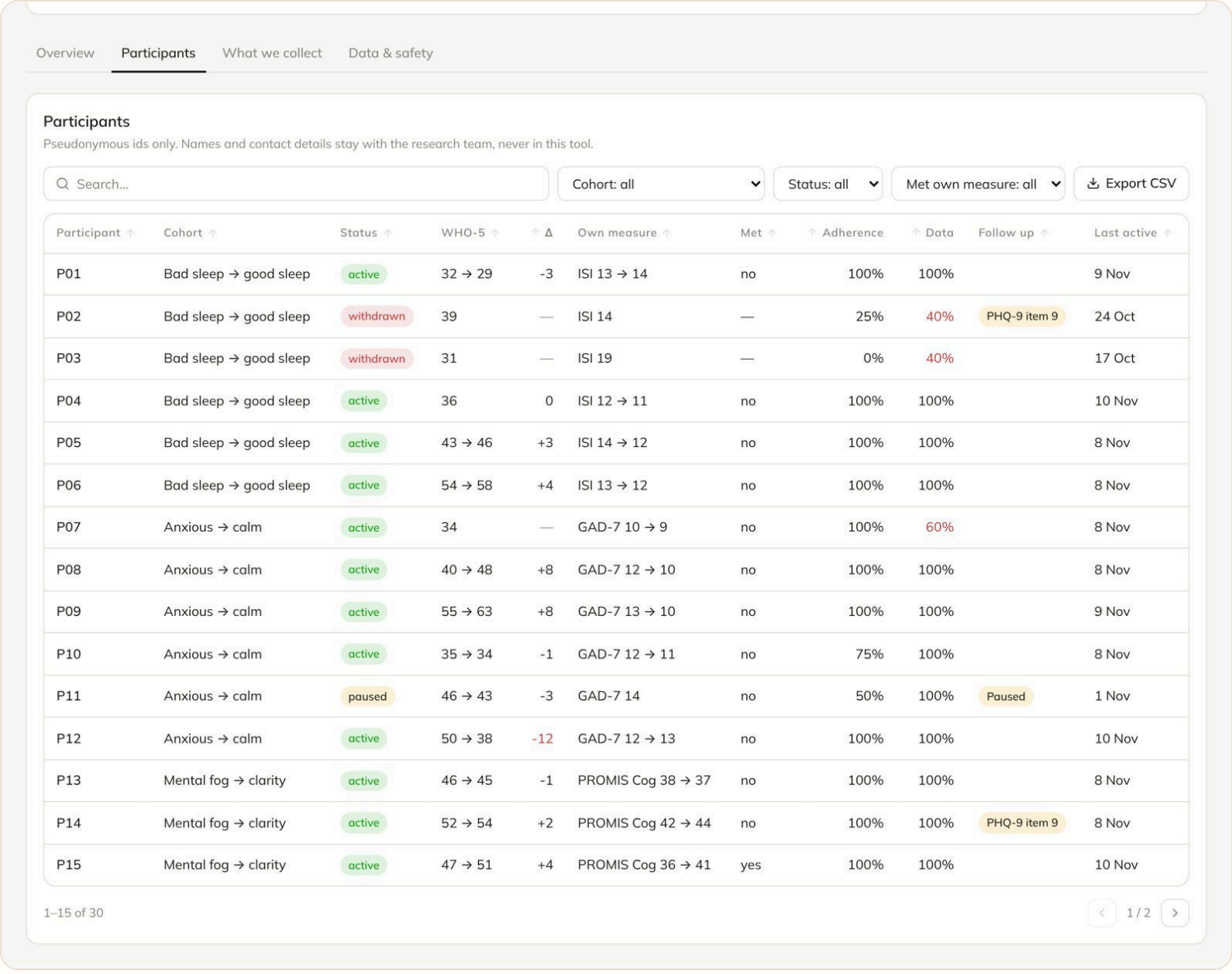
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9. The Experimentation section in admin

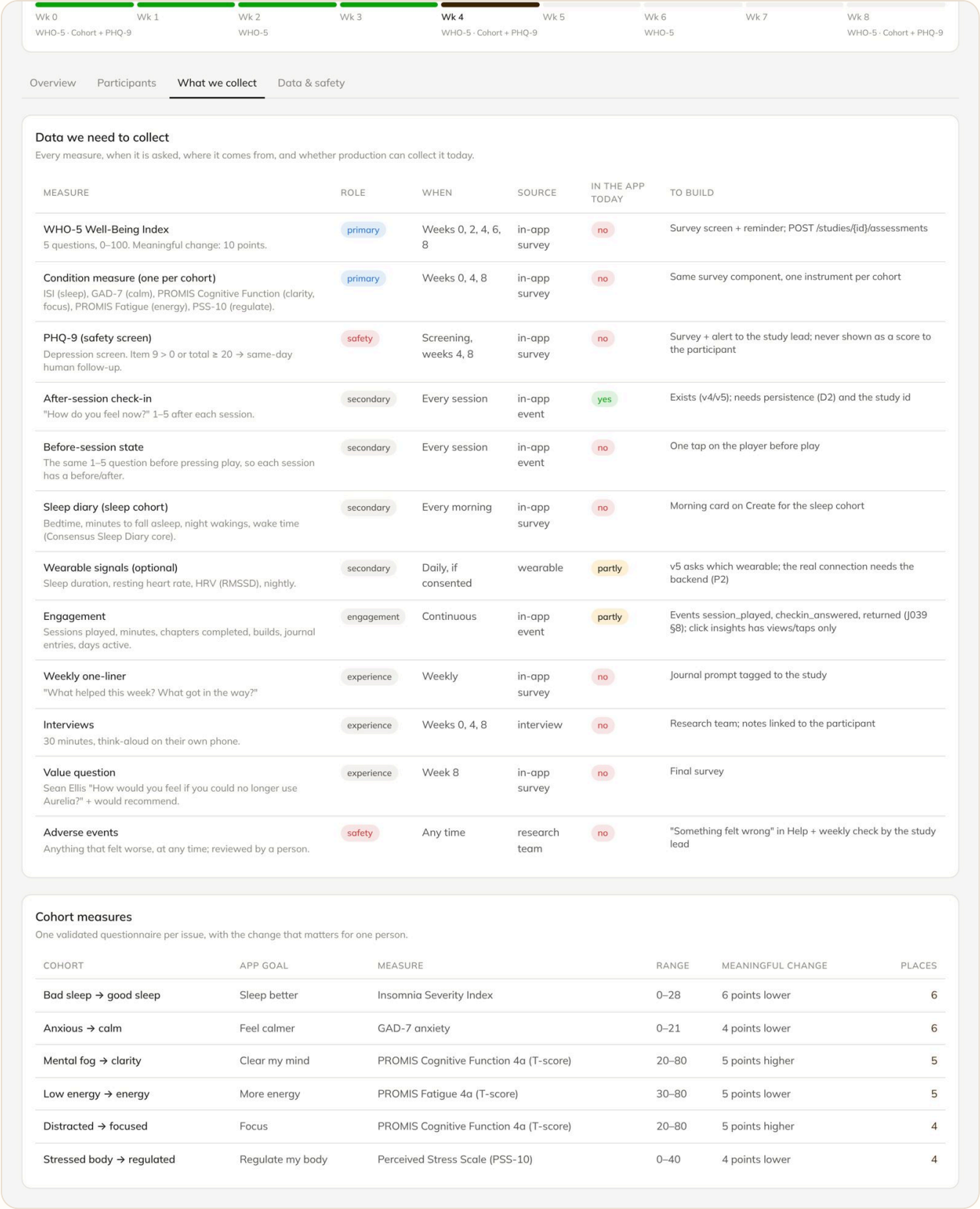
New in admin on staging: **Experimentation → Feedback group study**. It has four views and a page per participant. Until the study runs on the backend it shows sample data, generated by the same rules the real data will use. The screenshots below are mid-study (week 4 of 8) on sample data.



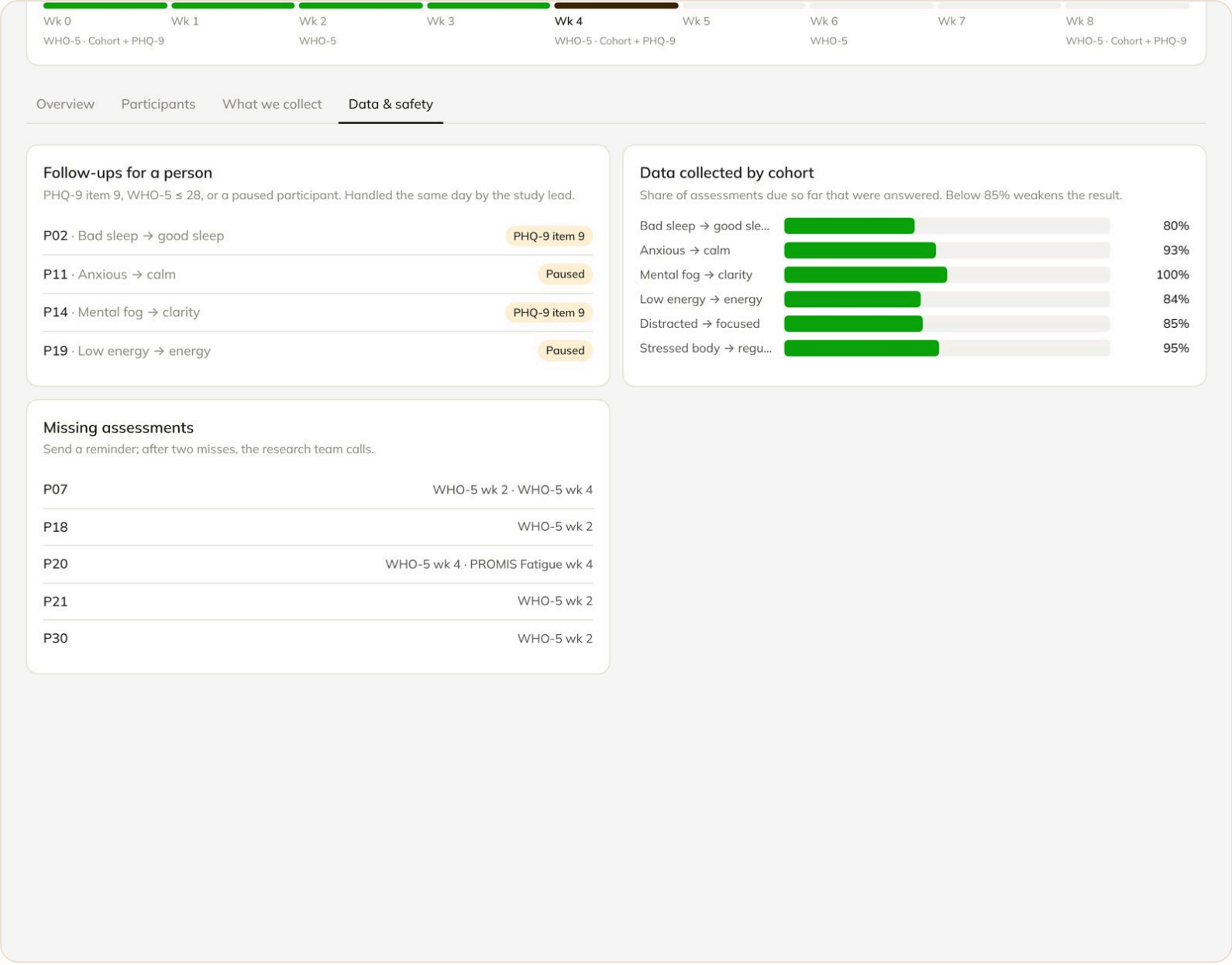
Overview. A timeline of the eight weeks and what is collected when; headline figures; the success criteria with their status (outcome lines "not yet" until week 8); well-being by cohort; a cohort table; and a box on what the study can and cannot show.



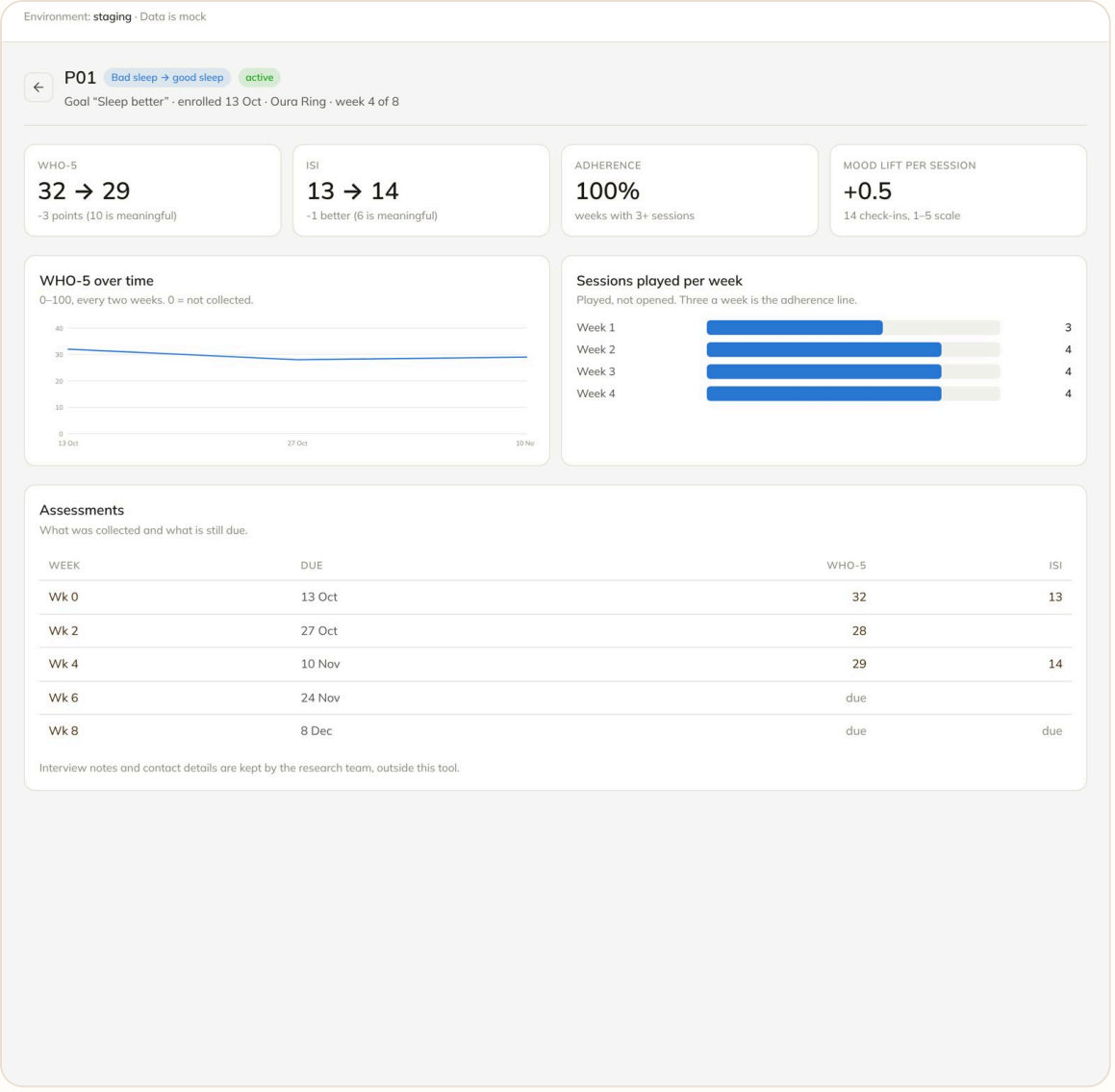
Participants. Thirty pseudonymous rows: cohort, status, WHO-5 before and now, their own measure, adherence, data completeness and follow-up flags. Filter by cohort, status or whether they met their measure; export to CSV.



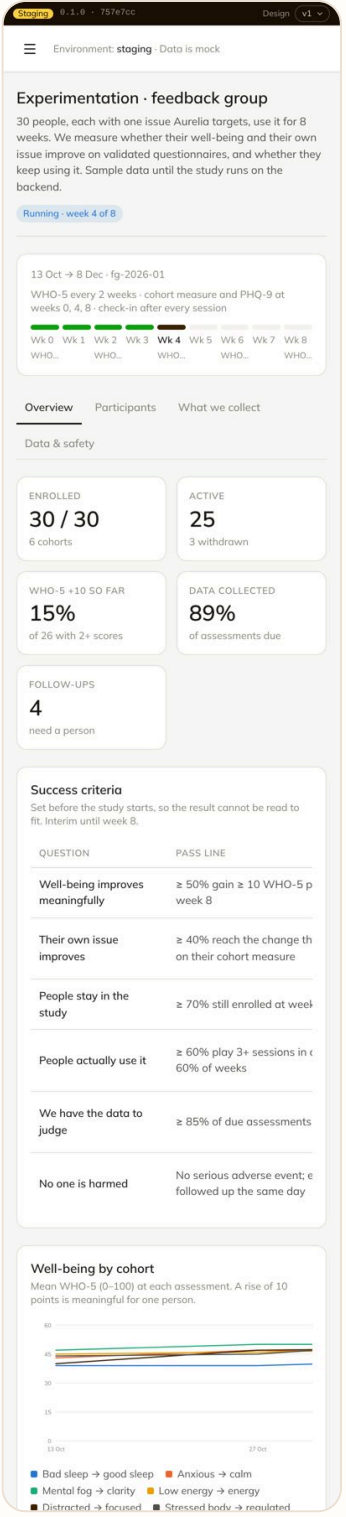
What we collect. The protocol from section 5 inside the tool, with what each item needs to be built, plus the cohort measures.



Data & safety. Follow-ups for a person, data collected per cohort, and every missing questionnaire, so nobody silently drops out of the data.



One participant. WHO-5 over time, their own measure, adherence, mood lift per session, sessions per week, and each questionnaire: done, missed or due. Contact details stay with the research team.



The same overview on a phone.

10. What must be built before week 0

ITEM	WHY	STATUS
Stay signed in, with data saved to the account (D2)	Eight weeks of check-ins cannot live in one browser	NEEDS BACKEND
Study enrolment and consent in the app	Links a person to fg-2026-01, a cohort and a pseudonymous id	TO BUILD
Questionnaire screen (WHO-5, cohort measures, PHQ-9) with reminders	The primary data; the API contract now includes it (§5.12)	TO BUILD
Before-play tap, sleep diary card, weekly one-liner	Secondary measures	TO BUILD
Events: session played, check-in answered, returned	Engagement and adherence	TO BUILD
Safety alert to the study lead	Same-day follow-up	TO BUILD
Wearable connection (Apple Health first)	Optional objective signal	OPTIONAL FOR WEEK 0
Admin Experimentation section	Tracking and measuring the group	ON STAGING (sample data)

If the backend is not ready, the study can start with the questionnaires in a form tool and the admin fed by a weekly import. That is slower and leaves more gaps, so it is a fallback, not the plan.

11. Decisions needed from you

1. Approve the design and pass lines

They must be fixed before the first person starts, or the result can be read to fit.

2. Who recruits, and with what incentive?

Recommended: two people per issue from outside the team, with free Pro for a year after completing.

3. A clinical advisor and an ethics review

To confirm the measures, the thresholds and the safety process.

4. Start date

Week 0 starts when the items in section 10 are ready; about three to four weeks of build after a yes.

AI analysis by Claude, based on its own analysis of the app and deep research. In the repository: retention simulation (docs/reviews/Aurelia-Retention-Simulation-2026-10-25.pdf), v5 proposal (docs/reviews/Aurelia-v5-Proposal-2026-10-27.pdf), API contract (docs/api/API-CONTRACT.md §5.12), admin rules (src/admin/data/study.ts).